OutOfNetworkNY

BY MADISON LAW FIRM PLLC

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The Out-of-Network Provider's Arbitration Readiness Guide

For physicians and practice administrators preparing out-of-network claims for New York IDR and federal No Surprises Act arbitration.

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ABOUT THIS GUIDE

How to use this guide

This guide explains how out-of-network payment disputes work in New York in 2026, and what a practice can do now to prepare its claims. It reflects the law and published fees as of November 5, 2026. Both systems changed significantly this year, and they will keep changing. Check current rules before relying on any figure here.

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Three rules that run through every chapter. (1) The patient's coverage decides the forum, not the payer's name or the service. (2) Billed charges are not the measure: the federal reviewer may not consider them, and the New York reviewer anchors on the usual and customary cost, so never value a claim on the bill. (3) Federal deadlines are counted in business days and are short; track them from the day the plan pays or denies.

This guide is general information, not legal advice about any claim. Reading it does not create an attorney-client relationship. Attorney Advertising. Prior results do not guarantee a similar outcome.

CHAPTER 01

Which process applies

Before anything else, sort each claim by how the patient is covered. Carriers such as Healthfirst, Fidelis, UnitedHealthcare, Aetna, and Cigna run commercial, self-funded, Medicaid, and Medicare lines, so the name on the EOB does not tell you which law applies.

Patient's coverage	Dispute process	Balance billing the patient
New York fully insured plan	New York IDR (FSL Art. 6), through the DFS portal	Not allowed for emergency services and surprise bills; patient owes in-network cost-sharing
Self-funded employer plan (ERISA), any state	Federal IDR (No Surprises Act)	Not allowed for covered services
Empire Plan / Student Employee Health Plan	New York IDR for disputes filed on or after Aug. 26, 2026 (State Plan rules)	Not allowed
Insured plan issued in another state	That state's process if it applies; otherwise federal IDR	Generally not allowed
Medicaid, fee-for-service or managed care	No IDR (managed care excluded from NY IDR for disputes filed on or after May 28, 2026)	Not allowed; Medicaid payment is payment in full
Medicare or Medicare Advantage	No IDR; Medicare rules apply (Chapter 9)	Only within Medicare limits
Uninsured or self-pay	Good-faith estimate rules; federal patient-provider dispute process	Limited by the good-faith estimate rules

How to tell fully insured from self-funded

- Read the card and EOB. Words like “self-funded,” “self-insured,” “administered by,” or “claims administrator” point to a self-funded plan. Large employers usually self-fund.
- Look for ERISA or Department of Labor appeal language on the EOB, which suggests an employer plan.
- Ask the plan. Its provider line can confirm the funding type for the member's group.
- Record the answer for every claim. Filing in the wrong forum wastes fees and can cost you the federal deadlines.

> For a fully insured New York plan, New York’s process generally applies *instead of* the federal one. The two are not alternatives you can choose between for the same claim.

CHAPTER 02

New York IDR

New York's Independent Dispute Resolution process was created by Financial Services Law Article 6 and is administered by the Department of Financial Services (DFS) under 23 NYCRR Part 400.

What it covers

- **Emergency services** by out-of-network providers for patients in fully insured New York plans.
- **Surprise bills**: services by an out-of-network physician at an in-network hospital or ambulatory surgery center where an in-network physician was unavailable, or the out-of-network physician provided services without the patient's knowledge, or unforeseen circumstances arose; and certain referrals by an in-network physician made without the patient's signed acknowledgment.

What it does not cover

- A patient who chose an out-of-network physician when an in-network one was available at the participating hospital or surgery center.
- Self-funded employer plans, Medicare, Medicare Advantage, and plans issued outside New York.
- Medicaid managed care disputes filed on or after May 28, 2026.

How a dispute is filed

1. The provider or plan enters the dispute in the DFS IDR portal. A dispute is "filed" when it is entered there.
2. DFS assigns a reviewer (an IDR entity, or IDRE) and a case number.
3. The provider sends the IDR application, supporting documents, and the full IDRE fee to the assigned entity.
4. For surprise bills, the patient's assignment of benefits (the DFS surprise bill certification form) is part of the filing.

The decision

The reviewer selects either the provider's fee or the plan's payment; it does not split the difference, though it may direct the parties to negotiate. The determination binds the provider, the plan, and the patient, and it is admissible in court.

The 2026 changes

Change	What it means
Medicaid managed care removed	Disputes filed on or after May 28, 2026 are ineligible (Part BB, Ch. 57, L. 2026).
State Plans added	Empire Plan and SEHP disputes filed on or after Aug. 26, 2026 are eligible. The reviewer picks the amount closer to the 50th percentile of allowed amounts; the plan cannot owe more than the 80th percentile.
Pay before review	Both sides pay the full IDRE fee up front: the filer with the application, the other side when it responds. The prevailing party is refunded within 30 days; settlements and split decisions are refunded pro rata.
Contract and prior-authorization bar	No fee can be awarded if the plan shows a contract with the provider or an affiliate for the same service at the same location, or a compliant prior-authorization notice identifying the service as out of network. The plan must raise this at the start.
Longer decision period	Generally 45 business days after the IDRE has all needed information (previously 30).

Sources: DFS Insurance Circular Letter No. 5 (2026); 23 NYCRR Part 400, Second Amendment (emergency, eff. Aug. 26, 2026).

Is there a filing deadline?

Article 6 itself does not set a filing deadline, but DFS's provider application (revised August 2026) requires the application to be sent to the assigned IDRE **within three years of the health plan's initial payment**. Treat that as the New York deadline. Claims past it may still support a breach-of-contract claim, generally subject to New York's six-year limitations period (CPLR 213(2)), which plan terms can shorten. Review the dates on every claim with counsel.

CHAPTER 03

Federal IDR, step by step

The No Surprises Act process (42 U.S.C. § 300gg-111; 45 C.F.R. § 149.510) covers self-funded employer plans in every state and insured plans not governed by a state process, for emergency services, non-emergency services by out-of-network providers at in-network facilities, and air ambulance.

Step	Deadline	What happens
1. Initial payment or denial	30 calendar days after a clean claim	The plan pays or denies. Remittance codes should show whether the claim is subject to the No Surprises Act.
2. Open negotiation	Start within 30 business days of the payment or denial; runs 30 business days	Required before IDR. Under the 2026 Operations Rule, the notice moves into the federal IDR portal and the other side must respond by the 15th business day.
3. Initiate IDR	Within 4 business days after negotiation ends	Either side files the notice of IDR initiation. Missing this window generally ends the claim.
4. IDRE selection and eligibility	A few business days	A certified IDR entity is selected and checks eligibility; parties may object to the entity or to eligibility.
5. Offers and fees	10 business days after IDRE selection	Each side submits a final offer, supporting information, and fees. Unpaid fees mean no offer is considered.
6. Decision	About 30 business days after IDRE selection	The IDRE selects one offer and explains why in writing.
7. Payment	30 calendar days after the decision	Any amount owed is paid. The prevailing party's IDRE fee is refunded.

Batching

Similar items or services against the same plan can be combined in one dispute. For disputes whose open negotiation begins on or after November 1, 2026, a batch can include up to 50 line items under expanded batching criteria (91 Fed. Reg. 33900; CMS implementation guidance, Aug. 7, 2026).

Cooling-off period

After a determination, the party that started IDR generally cannot bring the same party back for the same or similar item or service for 90 calendar days. Claims whose negotiation ends in that window can be filed within 30 business days after it ends.

> Federal deadlines are short and counted in business days. Build a tickler for every out-of-network claim from the day the plan pays or denies it. Most lost federal claims are lost on timing, not merits.

Several procedural changes in the 2026 Operations Rule take effect 90 days after CMS announces that the supporting portal function is live. Check the CMS No Surprises site for current applicability dates.

CHAPTER 04

What each reviewer weighs, and what it cannot

Both systems use pick-one (“baseball”) arbitration, but the reviewers weigh different evidence. Build each submission around the factors that reviewer is required to consider.

	New York IDR (FSL § 604)	Federal IDR (42 U.S.C. § 300gg-111(c)(5))
Main benchmark	Usual and customary cost: the 80th percentile of charges for the service by similar providers in the same geographic area, from FAIR Health data	The qualifying payment amount (QPA), the plan's median contracted rate, with no presumption in its favor
Other factors	Any gross disparity between the fee and what the plan pays similar providers; the physician's training, education, and experience; the physician's usual charge; case complexity and circumstances; patient characteristics	Provider training and experience; patient acuity and complexity; market share; teaching status, case mix, and scope of services; good-faith network contracting efforts; contracted rates from the prior four plan years
Not the measure	The bill itself. The physician’s usual charge is one factor among several; the usual and customary cost carries the analysis	Billed charges; usual and customary charges; Medicare, Medicaid, CHIP, and TRICARE rates
State Plans	50th percentile of allowed amounts benchmark, 80th percentile cap	Not applicable

The QPA after the 2026 litigation

The QPA is calculated by the plan. Providers have long challenged how it is computed, including the use of rates for services a provider never actually furnishes. On August 11, 2026, the Fifth Circuit, sitting en banc in *Texas Medical Association v. HHS*, vacated two of the three challenged QPA-methodology provisions of the 2021 rule; the mandate issued October 2, 2026. Expect further guidance. In a federal submission, question the QPA when the evidence supports it, and always present the additional circumstances.

FAIR Health in New York

FAIR Health is the benchmark database DFS designates for usual and customary cost. Pull the 80th percentile for the CPT code, modifier, and geographic area where the service was performed, as close to filing as practical, and keep a dated record of the lookup.

CHAPTER 05

Reading an EOB and spotting eligible claims

Eligible claims hide in remittance data. A short screen on each out-of-network claim separates the ones worth disputing from the ones that are not.

The screen

1. **Network status.** Were you out of network for this plan, for this service, at this location, on the date of service? In New York, a contract covering the same service at the same location (including through an affiliate) bars an award.
2. **Coverage.** Fully insured New York, self-funded, State Plan, out-of-state, Medicaid, or Medicare? (Chapter 1.)
3. **Setting.** Emergency, or a service at an in-network hospital or surgery center? Did the patient knowingly choose you when an in-network physician was available?
4. **Authorization.** Did a prior-authorization notice identify the service as out of network?
5. **Dates.** When did the plan pay or deny? For federal claims, is the open-negotiation window still open?
6. **Gap.** How far is the payment below the benchmark that reviewer will weigh, after process fees?

Fields to capture from each EOB or 835

- CPT/HCPCS code and modifiers; units; date of service; place of service
- Billed, allowed, and paid amounts; patient cost-sharing
- Adjustment codes (CARC/RARC), including any code indicating the claim is subject to the No Surprises Act
- Plan name, group number, and funding type; payment or denial date

> Look for patterns. When a plan pays one CPT code at the same low rate across many claims, those claims can often be prioritized together and, federally, batched.

CHAPTER 06

Documents to collect at intake

Disputes are often won or lost on paperwork collected at the front desk months earlier. Build these into intake rather than reconstructing them later.

1. **Insurance card, front and back,** and the plan's funding type.
2. **Assignment of benefits.** For New York surprise bills, the DFS surprise bill certification form signed by the patient.
3. **Disclosures.** Any New York out-of-network disclosure or referral acknowledgment, and any federal notice-and-consent forms (which ancillary physicians cannot use at in-network facilities).
4. **Prior-authorization correspondence,** including any notice describing the service as out of network.
5. **Clinical record:** operative report or notes showing complexity, time, and circumstances.
6. **Remittances:** the EOB or 835 for every claim.
7. **Credentials:** CV, board certification, fellowship training.
8. **Charge history:** your usual charges for the services, and prior contracted rates with the plan, if any.
9. **Negotiation record:** every offer and response, with dates.

HIPAA

Disclosing claim information to counsel to obtain payment is permitted, but your lawyer is a business associate. Sign a business associate agreement before sending any patient information, send only what is needed, and expect filings to be redacted.

CHAPTER 07

Setting your offer in pick-one arbitration

Because the reviewer picks one number, the offer that wins is the one the reviewer can most easily justify from the record, not the highest one you can defend.

- **Anchor to the right benchmark.** In New York, anchor to the usual and customary cost and the other § 604 factors. Federally, anchor to the QPA and the additional circumstances that justify more.
- **Do not anchor to billed charges.** The federal reviewer may not consider them, and in New York your usual charge is only one factor. An offer built on the bill invites the reviewer to pick the plan's number.
- **Explain the premium.** Tie any amount above the benchmark to specific facts: complexity, time, acuity, credentials, market conditions.
- **Be consistent.** Offers that move unpredictably across similar claims undermine credibility with the same reviewers.
- **Weigh settlement.** In New York the reviewer may direct the parties to negotiate, and settlement triggers pro rata fee refunds.

A worked example. If the reviewer's benchmark for a service is $14,000 and the plan offers $6,000, a $14,500 offer supported by the record is easier to select than a $25,000 offer based on the bill. These figures are illustrations, not predictions.

CHAPTER 08

Fees and costs in both systems

These are the process costs as published in November 2026. Your lawyer’s fee is separate and should be set out in a written engagement letter.

	New York IDR	Federal IDR
Government fee	None	$15 per party per dispute for disputes initiated on or after June 11, 2026 ($115 before). Not refunded.
Reviewer (IDRE) fee	$300 to $495, depending on the assigned entity: MCMC $300; Maximus $375; Kepro (IMEDECS) $395; HHC $425; ProPeer $495	Fixed fee set by each certified entity within federal ranges: $200 to $840 single; $268 to $1,173 for 2 to 25 batched line items; plus $75 to $250 per additional 25 line items
When paid	Both sides, up front (disputes on or after Aug. 26, 2026)	Both sides, with their offers
Who ends up paying	The losing side; the winner is refunded within 30 days	The losing side; the winner's IDRE fee is refunded
Ineligible or settled	Processing fee $100 to $175 kept if ineligible; settlement fee $175 to $300 with pro rata refunds	Administrative fee not refunded

Sources: DFS IDRE fee schedule (dfs.ny.gov); Federal IDR Operations Rule, 91 Fed. Reg. 33900 (June 4, 2026); federal IDR fee rule, 88 Fed. Reg. 88494 (Dec. 21, 2023). Each federal entity posts its fixed fees on the CMS certified IDRE list.

Doing the math

Before filing, compare the realistic gap on a claim (benchmark minus payment) with the reviewer fee you would bear if you lose. Low-dollar claims are often only worth pursuing federally in a batch, or in New York when the same evidence supports many similar disputes.

CHAPTER 09

Medicare and Medicaid: what is outside IDR

Neither New York IDR nor the No Surprises Act applies to Medicare, Medicare Advantage, Medicaid, CHIP, TRICARE, or VA coverage. Those programs protect patients in their own ways and set their own payment rules.

Original Medicare

Participating providers accept the Medicare-approved amount. A non-participating provider that does not accept assignment may charge no more than the limiting charge, which is 115% of the non-participating fee schedule amount.

Medicare Advantage

- A provider not contracted with the Medicare Advantage plan must accept what Original Medicare would have paid as payment in full, and may not bill the patient beyond plan cost-sharing (42 C.F.R. § 422.214).
- **Underpaid** (paid less than Original Medicare would have): file a payment dispute with the plan, then request review by CMS's independent Payment Dispute Resolution Contractor. Check the plan's deadlines.
- **Denied:** request reconsideration with a signed waiver of liability, which opens the Medicare appeals process.
- A contracted provider's remedy is its contract.

Medicaid

- A provider that accepts a patient's Medicaid must accept the Medicaid payment as payment in full; the patient cannot be balance billed.
- Medicaid managed care disputes filed on or after May 28, 2026 are excluded from New York IDR.
- Remaining routes: plan appeals, the provider contract's dispute terms, and complaints to the New York State Department of Health.
- The measure of an underpayment is the contract or Medicaid fee schedule rate, not billed charges.

Qualified Medicare Beneficiaries (QMBs) cannot be billed for Medicare deductibles, coinsurance, or copayments. Check dual-eligible status before sending any patient a bill.

CHAPTER 10

Collecting an award

Winning is not the same as being paid. Track every determination to payment.

New York

A New York IDR determination is binding on the plan and admissible in court (FSL §§ 605, 607). Article 6 does not say how an unpaid determination becomes a court judgment, and it does not refer to the CPLR Article 75 confirmation procedure. Escalate to DFS, and consider a suit in New York state court on the determination. For New York-regulated plans, the prompt pay law (Insurance Law § 3224-a) requires undisputed claims to be paid within 30 days of electronic submission (45 days on paper), with interest on late payments at the greater of 12% a year or the corporate tax rate.

Federal

Federal determinations are binding and can be vacated only on the narrow grounds in the Federal Arbitration Act. But the Fifth Circuit (*Guardian Flight v. Health Care Service Corp.*, 2025; cert. denied Jan. 12, 2026) and the Second Circuit (*East Coast Advanced Plastic Surgery v. Cigna*, Sept. 17, 2026) have held that the No Surprises Act gives providers no private right to sue to enforce an award. In New York federal courts, unpaid federal awards go to the regulators: CMS, the Department of Labor for employer plans, or the state insurance regulator. Other theories remain untested.

> Calendar the payment due date (30 calendar days federally) for every award, and escalate promptly when it passes.

CHAPTER 11

Common mistakes

Most lost claims are lost before the reviewer ever looks at the merits.

1. **Wrong forum.** Filing a self-funded plan claim in New York IDR, or a fully insured New York claim federally.
2. **Missed deadlines.** Federally, not starting open negotiation within 30 business days or IDR within 4 business days after it ends; in New York, filing more than three years after the plan's initial payment.
3. **Valuing on billed charges.** Federal reviewers may not consider them, and New York reviewers anchor on the usual and customary cost.
4. **Medicare and Medicaid claims in IDR.** They are not eligible. Medicaid managed care was removed from New York IDR in 2026.
5. **Missing intake paperwork.** No assignment of benefits or surprise bill form for a New York surprise bill.
6. **Ignoring the contract and prior-authorization bar.** In New York, an affiliate's contract or an out-of-network prior-authorization notice can defeat the claim.
7. **Unpaid fees.** Federally, unpaid fees mean your offer is not considered; in New York both sides now pay up front.
8. **Thin evidence.** No credentials, no complexity narrative, no benchmark data.
9. **Letting awards sit.** Not tracking payment after a win.
10. **Sending patient data without a BAA.**

Talk to us

Madison Law Firm PLLC represents New York physicians, groups, and surgery centers in New York IDR and federal No Surprises Act arbitration. For a free review of your out-of-network claims, call (212) 300-3191, email inquiries@madisonlawfirm.com, or visit outofnetworkny.com/case-evaluator. We handle surprise billing arbitration on a contingency basis under a written engagement letter.

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